



Document Management System



Sultanate of Oman
Ministry of Health
The Royal Hospital
Department of Child Health

Code: CH-PICU-POL-10-Vers.1.0

Effective Date: 19/02/2024

Target Review Date: 20/02/2027

Title: Pediatric High Dependency Admission Policy

Pediatric High Dependency Admission Criteria

Any patient admitted to High Dependency, consultant to be informed morning/ on call.

I. GENERAL:

- Age: Below 13 year old.
- Weight: Equal or more than 2 kg.
- DNR Patients should not be admitted in high dependency (PHD) unit. **EXCEPT** if reversed temporary for medical reason or DKA requiring insulin infusion.
- Patients with airborne infection suspected or confirmed (eg. Chicken pox, TB etc) should not be admitted in PHD.
- Fever and Rash should not be admitted in PHD.
- **No patient should be admitted routinely as temporary recovery after procedure** such as MRI, line insertion, bronchoscopy etc. Unless meeting any of admission criteria mentioned above.

II . RESPIRATORY SYSTEM:

Patients with moderate and non-life-threatening pulmonary or airway disease, but not limited to:

- Required minimal ventilator support via mature and stable tracheostomy.
- Patients who are on any type of ventilation requiring higher ventilation settings or oxygen than their baseline but stable hemodynamically.
- Rapidly progressive pulmonary disease of moderate severity with risk of progression to respiratory failure and/or total obstruction.
- Requirement of supplemental oxygen more than 1L, or in distress, **Except** patients with chronic lung disease or on home oxygen if requires more than 3 L of supplemental oxygen.
- Patients with mature tracheostomy requiring suction less than 3 hours; if care giver is not trained.
- Patient requiring suction 2 hourly or less, or continuous nebulized medications or magnesium infusion.
- Patients requiring apnea work up and cardiorespiratory monitoring.
- Patients requiring non-invasive ventilatory support (HFNC,CPAP or BiPAP)

III. CARDIOVASCULAR SYSTEM:

- Patients with non-life-threatening cardiac disease requiring low dose inotropic or vasodilator therapy.
- Inotropes requirement for patients who are hemodynamically stable:
 - a. Dopamine or Dobutamine equal or less than 10 mcg/kg/min.
 - b. Nor-epinephrine or epinephrine equal or less than 0.05 mcg/kg/min.
 - c. One inotrope at a time, if on two inotropes or more than the dose written above should be transferred to PICU, except in bed crisis.
 - d. Any patient started on inotrope PICU team needs to be alerted only.

IV. NEUROLOGIC:

Patients with non-life-threatening neurologic disease, conditions include but are not limited to:

- Seizures, responsive to therapy but requiring continuous cardiorespiratory monitoring, with no hemodynamic compromise, but potential for respiratory compromise.
- Patient with altered sensorium in whom neurologic deterioration or depression is unlikely, but require frequent neurologic assessment, with GCS more than 10.
- Acute inflammation or infections of the spinal cord, meninges, or brain with neurologic deficiency or other complications.
- Progressive neuromuscular dysfunction without altered sensorium requiring cardio-respiratory monitoring.
- Requiring sedatives for patients whose airway is maintained:
 - a. Midazolam **NOT** more than 1 mcg/kg/min.
 - b. Fentanyl **NOT** more than 1 mcg/kg/hr.
 - c. Ketamine **NOT** more than 10 mcg/kg/min.
 - d. Dexmedetomidine (Precedex) **NOT** more than 0.5 mcg/kg/hr.
 - e. **Only one** sedative at a time, except patient transferred from PICU with weaning doses of sedatives for withdrawal score less than 5.

V. HEMATOLOGY / ONCOLOGY:

Patients with non-life-threatening conditions, include, but are not limited to:

- Requiring non-invasive ventilation and /or inotropic support with the above mentioned criteria for inotrope.
- Acute hemolysis if unstable hemodynamically.

VI. ENDOCRINE / METABOLIC:

Patients with non-life-threatening and potentially unstable endocrine or metabolic disease. Conditions include, but are not limited to:

- DNR patients with DKA requiring insulin infusion are eligible for HD admission.
- Hyperkalemia less than 6 mmol/L and not persistent, requiring cardiac monitoring and acute intervention.
- Severe hyponatremia between 120 to 125 or hypernatremia between 155 to 165, with-out seizures or altered sensorium GCS more than 13.
- Hypo or hyperglycemia requiring invasive monitoring.
- Persistent hypo or hypercalcemia requiring calcium infusion.
- Moderate metabolic acidosis requiring bicarbonate infusion.
- Inborn error of metabolism with acute deterioration requiring cardiorespiratory monitoring.

VII. GASTROINTESTINAL SYSTEM:

Patients with non- life-threatening or unstable gastrointestinal disease.

Conditions include, but not limited to:

- Acute gastrointestinal bleeding without hemodynamic or respiratory compromise.
- Emergency endoscopy for removal of foreign bodies with no cardiorespiratory compromise.
- Chronic gastrointestinal or hepatobiliary insufficiency without coma, hemodynamic or respiratory instability.

VIII. RENAL SYSTEM:

- Hypertension without seizures or encephalopathy who require close monitoring and frequent intermittent intravenous or oral medications and blood pressure monitoring.

IX. OTHERS

Patients with non-life-threatening and potentially unstable multi system disease. Conditions include, but not limited to:

- Uncomplicated toxic ingestions and drug overdose with no cardiorespiratory compromise, which require close monitoring.
- Post pleural drain after initial stabilization.
- No arterial line to be inserted in Paediatric HD except:
 - If blood sampling required every four hourly and patient has no functional central line.
 - If arterial line inserted it should not stay more than 48 hours.
 - Arterial lines should not be used for routine sampling.

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Review Details

	Review BY	Review Date
1	PICU Team	31/05/2020

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